BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

Check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver	Baseline bloods; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds	blood pressure (BP), urine protein and wound/procedure review	BP before and after treatment; urine protein	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan
Tablets / hands and feet	Confirm written dose, tablet supply and hold instructions	Review adherence, mouth, bowel and skin symptoms	Days 1–14 tablets; Days 15–21 planned break unless team changes it	Do not restart held tablets or replace missed doses without advice	Return unused tablets as instructed; follow persistent symptoms
Heart checks	Cardiac history and examination; consider an echocardiogram after chest-wall radiotherapy or prior anthracycline. Further tests depend on symptoms and the clinical plan.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
international normalised ratio (blood-clotting test) (INR) if taking warfarin	Check anticoagulant and baseline INR	If taking warfarin: INR weekly until the warfarin dose is stable, then before each cycle.	Report bleeding or bruising	Adjust warfarin only as prescribed	Continue the agreed anticoagulation plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00623 v3a · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Bevacizumab + capecitabine: treatment and checks

Advanced colorectal cancer

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00623	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00623	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds NCCP 00623	BP, urine protein and wound/ procedure review	BP before and after treatment; urine protein	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Tablets / hands and feet NCCP 00623	Confirm written dose, tablet supply and hold instructions	Review adherence, mouth, bowel and skin symptoms	Days 1–14 tablets; Days 15–21 planned break unless team changes it	Do not restart held tablets or replace missed doses without advice	Return unused tablets as instructed; follow persistent symptoms
Heart checks NCCP 00623	Cardiac history and examination; consider an echocardiogram after chest-wall radiotherapy or prior anthracycline. Further tests depend on symptoms and the clinical plan.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00623	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
INR if taking warfarin NCCP 00623	Check anticoagulant and baseline INR	If taking warfarin: INR weekly until the warfarin dose is stable, then before each cycle.	Report bleeding or bruising	Adjust warfarin only as prescribed	Continue the agreed anticoagulation plan

What each check is for

1 Confirm your personal plan

Before starting

This combination treats metastatic or unresectable colorectal cancer when an irinotecan- or oxaliplatin-based combination is unsuitable. Its aims may include cancer control, symptom relief and longer survival.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00623

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32